Mr. Om prakash

Age / Sex : 56 YRS / M

Referred by : SHRI SHYAM POLYCLINIC AND HOSPITAL

Reg. no. : 6984

6984

Registered on : 20/10/2026 08:54 PM

Collected on : 20/10/2026 08:00 PM

Received on : 20/10/2026 08:02 PM

Reported on : 20/10/2026 09:01 PM

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HAEMATOLOGY

COMPLETE BLOOD COUNT (CBC)

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN		15.9	g/dl	13 - 17
TOTAL LEUKOCYTE COUNT	**H**	**18,000**	**cumm**	**4,800 - 10,800**
DIFFERENTIAL LEUCOCYTE COUNT				
NEUTROPHILS	**H**	**81**	**%**	**40 - 80**
LYMPHOCYTE	**L**	**15**	**%**	**20 - 40**
EOSINOPHILS		03	%	1 - 6
MONOCYTES	**L**	**01**	**%**	**2 - 10**
BASOPHILS		00	%	< 2
PLATELET COUNT	**L**	**1.27**	**lakhs/cumm**	**1.5 - 4.1**
TOTAL RBC COUNT		4.96	million/cumm	4.5 - 5.5
HEMATOCRIT VALUE, HCT		41.5	%	40 - 50
MEAN CORPUSCULAR VOLUME, MCV		83.7	fL	83 - 101
MEAN CELL HAEMOGLOBIN, MCH	**H**	**32.1**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**38.3**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV	**H**	**12.9**	**fL**	**6.5 - 12**
R.D.W. - SD		39.4	fL	39 - 46
R.D.W. - CV		13.7	%	11.6 - 14
P-LCR	**H**	**49.2**		**19.7 - 42.4**
P.D.W.	**H**	**16.2**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters:

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

TEST	VALUE	UNIT	REFERENCE
BLOOD GROUP & RH.			
ABO	O		
RH (ANTI -D)	POSITIVE		

SEROLOGY & IMMUNOLOGY

VIRAL MARKER

TEST	VALUE	UNIT	REFERENCE
HIV (CARD TEST)			
HIV - 1	NEGATIVE		
HIV - 2	NEGATIVE		

HEPATITIS C VIRUS, HCV	NEGATIVE
HBSAG	NEGATIVE

HbsAg is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

Anti HCV card test interpretation

Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

HIV Interpretation

A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

BIOCHEMISTRY

BILIRUBIN TOTAL, DIRECT & INDIRECT

TEST	VALUE	UNIT	REFERENCE
SERUM BILIRUBIN (TOTAL)	0.86	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)	0.12	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)	0.74	mg/dl	0.2 - 1

TEST	VALUE	UNIT	REFERENCE
SERUM CREATININE	1.01	mg/dl	0.72 - 1.43

Creatinine is a waste product produced by muscles from the breakdown of a compound called creatine. It is removed from the body by the kidneys, which filter almost all of it from the blood and release it into the urine.
Creatine is part of the cycle that produces energy needed to contract muscles. Both creatine and creatinine are produced by the body at a relatively constant rate. Since almost all creatinine is filtered from the blood by the kidneys and released into the urine, blood levels are usually a good indicator of how well the kidneys are working.

Higher than normal level may be due to:
- Blockage in the urinary tract
- Kidney problems, such as kidney damage or failure, infection, or reduced blood flow
- Loss of body fluid (dehydration)
- Muscle problems, such as breakdown of muscle fibers
- Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia)

Lower than normal level may be due to:
- Myasthenia Gravis
- Muscular dystrophy.

Low serum creatinine values are rare. They almost always reflect low muscle mass.

SERUM UREA	38.57	mg/dl	18 - 55

SGPT (ALT)	35.11	U/l	13 - 40

Physiological basis
Intracellular enzyme involved in amino acid metabolism. Present in large concentrations in liver, kidney; in smaller amounts, in skeletal muscle and heart. Released with tissue damage, particularly liver injury.

Interpretation
Increased in: Acute viral hepatitis (ALT > AST), biliary tract obstruction (cholangitis, choledocholithiasis), alcoholic hepatitis and cirrhosis (AST > ALT), liver abscess, metastatic or primary liver cancer; nonalcoholic steatohepatitis; right heart failure, ischemia or hypoxia, injury to liver (“shock liver”), extensive trauma; drugs that cause cholestasis or hepatotoxicity.
Decreased in: Pyridoxine (vitamin B6) deficiency.

Comments
ALT is the preferred enzyme for evaluation of liver injury.

SGOT (AST)	30.68	U/l	0 - 37

Physiological basis:
Intracellular enzyme involved in amino acid metabolism. Present in large concentrations in liver, skeletal muscle, brain, red cells, and heart. Released into the bloodstream when tissue is damaged, especially in liver injury.

Interpretation
Increased in: Acute viral hepatitis (ALT > AST), biliary tract obstruction (cholangitis, choledocholithiasis), alcoholic hepatitis and cirrhosis (AST > ALT), liver abscess, metastatic or primary liver cancer; right heart failure, ischemic or hypoxic injury to liver (“shock liver”), extensive trauma. Drugs that cause cholestasis or hepatotoxicity.
Decreased in: Pyridoxine (vitamin B6) deficiency

Comments
Test is not indicated for diagnosis of myocardial infarction.
AST/ALT ratio >1 suggests cirrhosis in patients with hepatitis C

RANDOM BLOOD SUGAR	124.56	mg/dl	70 - 140

SGOT/SGPT Method: Calculated	0.87

~~~ End of report ~~~

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